

Insulin Administration Protocol

Internal policy / SOP - for demo purposes only

1. Purpose

This protocol governs the administration of insulin and the management of glycemic events for residents diagnosed with diabetes mellitus. It applies to licensed nursing staff authorized to administer insulin.

2. Authorized Insulin Orders

Insulin shall only be administered pursuant to a current written or electronic order from the resident's attending physician. Standing orders are not permitted. The order must specify the insulin type (e.g., glargine, aspart, regular), dose, route, timing, and the blood-glucose ranges for sliding-scale corrections if applicable.

3. Pre-Administration Verification

Before each dose the administering nurse must verify the five rights: right resident, right medication, right dose, right route, right time. The resident's blood glucose must be measured within 15 minutes of any meal-time or correction dose, and the result documented in the MAR. For mealtime insulin, do not administer if the resident's meal will be delayed by more than 15 minutes or if the resident is NPO.

4. Administration Technique

Insulin is administered subcutaneously using a rotating site schedule (abdomen, thigh, upper arm, buttock). Sites must be rotated at every dose to prevent lipohypertrophy. Use a new needle for each injection. Discard sharps immediately into the bedside sharps container.

5. Hypoglycemia Management

Hypoglycemia is defined as a blood glucose below 70 mg/dL. The 'Rule of 15' applies: administer 15 grams of fast-acting carbohydrate (4 oz juice, 4 glucose tabs, or 1 tablespoon honey), recheck blood glucose in 15 minutes, and repeat once if still below 70. If the resident is unresponsive or unable to swallow, administer IM glucagon and notify the attending physician immediately. All hypoglycemic events must be documented and reported to the medical director within 24 hours.

6. Hyperglycemia Management

Blood glucose above 250 mg/dL on two consecutive readings requires notification of the attending physician. Readings above 400 mg/dL, or any glucose with associated symptoms of DKA (nausea, vomiting, abdominal pain, fruity breath), require immediate physician notification and consideration for hospital transfer.

7. Eligibility for Self-Administration

A resident may be approved for self-administration of insulin only after an interdisciplinary evaluation including cognitive screening, demonstrated technique competency, and physician order. Approval is reviewed every 90 days.

8. Documentation

Each insulin dose, blood-glucose reading, and any hypoglycemic or hyperglycemic event must be documented in the medication-administration record (MAR) within one hour of administration. Missed doses or refusals must be documented with the reason and reported to the charge nurse.